After Visit Summary: Discharge Instructions

You were cared for at Riverbend General Hospital from **10/08/2026** to **10/12/2026**. Please read these instructions and bring this paper to all of your follow-up visits.

Patient	Jordan Miller, 67-year-old male	Admitting unit	Cardiac Step-Down, 4 West
Attending physician	Maria Chen, MD (Hospital Medicine)	Consulting service	Cardiology: A. Rivera, MD
Allergies	No known drug allergies (NKDA)	Code status	Full code
Discharged to	Home with spouse	Primary care	Samuel Okafor, MD

Why you were in the hospital

Principal diagnosis: Acute on chronic heart failure with reduced ejection fraction (HFrEF), NYHA class III.
Other diagnoses: Hypertension; hypokalemia (low potassium), resolved; chronic kidney disease stage 2.
What this means: Your heart is not pumping as strongly as it should. Fluid built up in your lungs and legs, which made you short of breath. In the hospital we removed extra fluid with medicine through your IV, and your breathing improved.

Hospital course (summary for your care team)

67M w/ HFrEF (TTE 10/09/2026: LVEF 30%, mod MR), HTN, CKD2, admitted with 1 wk progressive DOE, orthopnea, and 3+ BLE edema. BNP 1,480. CXR: pulmonary vascular congestion, small bilateral effusions. Diuresed with IV furosemide 40 mg BID; net negative 4.2 L. Weight 84.1 kg on admission, 80.0 kg at discharge. K 3.2 on 10/10, repleted; started KCl. Amlodipine discontinued in favor of GDMT. Metoprolol succinate started 10/11 and tolerated (HR 78, BP 118/72). Cr stable at 1.1. Ambulating independently on room air (SpO2 96%).

Your medicine list

This is your complete list of medicines. **Changes from before your hospital stay are marked.** Throw away or set aside any old bottles that do not match this list, and bring all your medicine bottles to your follow-up visits.

Medicine	How to take it	What changed
furosemide (LASIX) 40 mg tablet	Take 1 tablet (40 mg) by mouth once daily in the morning. Sig: 40 mg PO QAM	**CHANGED**: dose increased. Previously 20 mg once daily.
metoprolol succinate ER (TOPROL XL) 25 mg 24 hr tablet	Take 1 tablet by mouth once daily in the evening. Swallow whole or split along the score line; do not crush or chew. Sig: 25 mg PO QPM	**NEW**
potassium chloride ER (KLOR-CON) 20 mEq tablet	Take 1 tablet by mouth once daily with breakfast and a full glass of water. Swallow whole. Sig: 20 mEq PO daily with food	**NEW**
lisinopril (ZESTRIL) 10 mg tablet	Take 1 tablet by mouth once daily in the morning. Sig: 10 mg PO daily	CONTINUE: no change
acetaminophen (TYLENOL) 500 mg tablet	Take 1 to 2 tablets by mouth every 6 hours as needed for pain. Do not take more than 3,000 mg in 24 hours. Sig: 500–1,000 mg PO q6h PRN pain; max 3 g/day	CONTINUE: no change
amlodipine (NORVASC) 5 mg tablet	**STOP taking this medicine.**	**STOPPED**

Pain relief: Do not take ibuprofen (Advil, Motrin) or naproxen (Aleve). These can cause fluid build-up and make heart failure worse. For pain, use acetaminophen (Tylenol) as directed above.

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Taking care of yourself at home

Daily weight	Weigh yourself every morning after using the bathroom and before breakfast, wearing similar clothes. Write it down in the log on page 3. Your discharge weight was **176 lb (80.0 kg)**.
Salt (sodium)	Limit sodium to less than **2,000 mg per day**. Read food labels. Avoid canned soups, deli meats, frozen dinners, and fast food.
Fluids	Limit all fluids to **2 liters per day** (about 8 cups). This includes water, coffee, soup, ice, and gelatin.
Activity	Walk every day, starting with 5 to 10 minutes and adding a few minutes each day as you feel able. No heavy lifting over 10 lb for 1 week. Rest if you feel short of breath.
Alcohol and smoking	Do not smoke. Ask your care team about alcohol.

When to call your care team

Call the **Cardiology Nurse Line at (555) 010-0142** (Monday to Friday, 8 a.m. to 5 p.m.) or the **after-hours line at (555) 010-0199** if you:

- gain more than **2 pounds in one day** or **5 pounds in one week**
- have more swelling in your legs, ankles, or feet
- feel dizzy or light-headed when standing
- need more pillows to breathe comfortably at night
- have a new cough that will not go away, or feel more tired than usual

CALL 911 for chest pain, severe shortness of breath, fainting, or coughing up pink, foamy mucus. Do not drive yourself.

About your heart failure

Heart failure is a long-term condition. Taking your medicines every day, weighing yourself, and limiting salt and fluids can help keep fluid from building up again and keep you out of the hospital. Your medicines may be adjusted at your follow-up visits.

Education provided

Heart failure self-care; daily weights; low-sodium diet; fluid restriction; medication changes; warning signs. Teach-back completed with patient and spouse on 10/12/2026.

Your follow-up appointments

What	When	Where / with whom
Blood test: potassium and kidney function (BMP)	Monday, 10/19/2026, any time 7 a.m. to 4 p.m.	Riverbend Outpatient Lab, 1st floor. No appointment needed; do not need to fast.
Cardiology follow-up	Tuesday, 10/20/2026 at 10:30 a.m.	A. Rivera, MD, Riverbend Heart Center, Suite 300. Bring your weight log and all medicine bottles.
Primary care follow-up	Within 7 days of discharge (please call to schedule)	Samuel Okafor, MD, Harbor Family Medicine, (555) 010-0160.

Tests still pending at discharge

None.

Daily weight log

Date	Weight (lb)	Notes
10/13		
10/14		
10/15		
10/16		
10/17		
10/18		
10/19		
10/20		

Important phone numbers

Cardiology Nurse Line (weekdays 8 a.m. to 5 p.m.)	(555) 010-0142
After-hours line	(555) 010-0199
Riverbend Pharmacy	(555) 010-0175
Emergency	911

I have received these instructions and my questions have been answered.

Patient or caregiver signature: _____	Date: _____
Electronically signed by: Maria Chen, MD, Hospital Medicine, 10/12/2026 11:42 a.m.	Nurse: K. Patel, RN

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